

Patient Name: [REDACTED]
MRN: [REDACTED]
DOB: [REDACTED] Sex: Female
Account Number: [REDACTED]
Visit Date:
Discharge date:
Patient Type: Inpatient
Location:
Attending Physician: [REDACTED]

Final Pathology Diagnosis Report

Accession Number: Senior Pathologist:

Collected Date/Time:
Received Date/Time:
Verified Date/Time:

Final Pathologic Diagnosis

THYROID (TOTAL THYROIDECTOMY):

- Papillary thyroid carcinoma, follicular variant (3.3 cm).
- Tumor focally extends to cauterized posterior resection margin.
- See synoptic report below.

TNM Stage: pT2 Nx Mx

ICD-O-3

Carcinoma, follicular variant 8340/3

Site: thyroid, NOS C 73.9

2/14/11 lw

SYNOPTIC REPORT

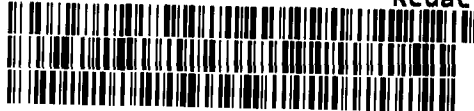
MACROSCOPIC

Specimen Type

Total thyroidectomy

UUID: E7412EA8-CEE7-4EA9-932F-33751A312E4B
TCGA-DO-A1JZ-01A-PR

Redacted



Tumor Site

Left lobe

Tumor Focality

Unifocal

Tumor Size (largest nodule)

Greatest dimension: 3.3 cm

*Additional dimensions: 3.0 x 2.5 cm

Criteria	Yes	No
Diagnosis Discrepancy	☒	☒
Primary Tumor Site Discrepancy	☒	☒
HIPAA Discrepancy	☒	☒
Prior Malignancy History	☒	☒
Dual/Synchronous Primary noted	☒	☒
Case Is (circle):	QUALIFIED	DISQUALIFIED
Review/Initials:	[Signature]	[Signature]
Date Reviewed:	2/14/11	

MICROSCOPIC

Histologic Type (check all that apply; choose 1 histologic type and all applicable subtypes)

Papillary carcinoma, Follicular variant

Pathologic Staging (pTNM)

Primary Tumor (pT)

pT2: Tumor more than 2 cm, but not more than 4 cm, limited to thyroid

Printed: [REDACTED]

Confidential Information

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Regional Lymph Nodes (pN)

pNX: Cannot be assessed

Distant Metastasis (pM)

pMX: Cannot be assessed

Margins

Margin involved by carcinoma
Site of involvement: posterior

***Venous/Lymphatic (Large/Small Vessel) Invasion (V/L)**

(Vascular invasion outside tumor or in capsule) is present

***Additional Pathologic Findings (check all that apply)**

* ☐ X Nodular goiter

I attest I have personally reviewed the specimen/slides and agree with the above findings.

Electronic Signature
Verified:

Resident:

Special Procedures

IMMUNOHISTOCHEMICAL STAINS:

Stains performed at the [REDACTED]

Representative tissue present in all sections examined.

Positive and negative controls react appropriately.

Printed:

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GALECTIN 3: Negative.
HBME: Negative.

This test was developed and its performance characteristics determined by the [REDACTED] or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high complexity clinical laboratory testing. *It has not been cleared*

Clinical History

-year-old pregnant female FNA biopsy showed papillary carcinoma. Total thyroidectomy.

Pre/Post-operative Diagnosis

Papillary thyroid carcinoma.

Gross Anatomic Description

(Dictated by Dr. [REDACTED] on [REDACTED]; Reviewed by Dr. [REDACTED])

Specimen received in one container.

Specimen: Designated "total thyroidectomy (single short superior - left lobe; double short superior - right lobe; single long - left lateral)" is received fresh for Tumor Bank labeled with the patient's name, [REDACTED] and "total thyroid". Specimen consists of a total thyroidectomy specimen weighing 44 grams with a left lobe measuring 5.3 x 4.8 x 2.5 cm, a right lobe measuring 4.5 x 2.5 x 1.3 cm connected by an isthmus measuring 1.0 x 0.4 x 0.1 cm. Normal tissue from right upper lobe is submitted for Tumor Bank and tumor specimen from left lower lobe is submitted for Tumor Bank. No frozen sections are performed. The external surface is smooth. On the left lower lobe, there is a protruding nodularity measuring 3.3 x 3.0 x 2.5 cm. On cut surface, the nodularity is encapsulated and well circumscribed with a homogeneous white/tan parenchyma. The right lobe of the thyroid comprises predominantly of gelatinous reddish-brown parenchyma within multiple ill-defined nodules. The mass is well encapsulated and abuts the inked surface but does not extend beyond the capsule. The uninvolved left thyroid parenchyma is beefy red and unremarkable. Photographs are taken.

Inking code: Black - anterior left; Green - anterior right; Blue - entire posterior specimen.

Section code: A1 - superior pole, left thyroid; A2 - superior extent of nodule; A3-A4 - full thickness section, superior portion of nodule from medial to lateral; A5-A7 - full thickness slice of mass at area of maximum diameter; A8-A12 - inferior pole, left thyroid, sections perpendicular to medial slices; A13 - isthmus; A14 - superior pole, right lobe; A15 - midportion of right

Printed:

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thyroid; A16 – inferior pole, right thyroid; A17-A30 – remainder of nodule entirely submitted.

Printed:

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